

STATEMENT OF MEDICAL NECESSITY

Velunexa (velunekimab) 150 mg/mL · Submitted to Anvara Health Plan

Patient Marcus Ellison · DOB 1971-06-18 · Male
Member ID AVH884210277 · Anvara Health Plan · Anvara Choice PPO
Requested product Velunexa (velunekimab) 150 mg/mL · J-code J3592
Regimen 150 mg subcutaneous, weeks 0 and 4, then every 12 weeks
Diagnosis L40.0 · Psoriasis vulgaris (moderate-to-severe plaque)

DIAGNOSIS AND HISTORY

Marcus Ellison is a 54-year-old man with a 26 month history of moderate-to-severe plaque psoriasis, diagnosed in June 2024 and managed continuously in this practice since that time. Involvement is widespread across the trunk, extensor surfaces of both arms, and both shins.

DISEASE SEVERITY

At today's visit body surface area involvement measures 18 percent and the PASI score is 16.4. The patient reports persistent pruritus interfering with sleep on most nights and has withdrawn from swimming, which he previously did three times a week. The Dermatology Life Quality Index is 17.

PRIOR THERAPY

The patient completed a trial of methotrexate 20 mg orally once weekly with folic acid supplementation, initiated 2025-11-04 and continued through 2026-02-24, a total of 16 weeks at a therapeutic dose. Adherence was verified through pharmacy refill history. PASI at week 16 was 15.9 against a pre-treatment PASI of 16.8, representing no clinically meaningful improvement. Methotrexate was discontinued for lack of efficacy, not for intolerance or cost.

Prior to systemic therapy the patient used clobetasol propionate 0.05 percent ointment and calcipotriene 0.005 percent for a combined nine months, and completed 24 sessions of narrowband UVB phototherapy between March and June 2025 with partial and non-durable response.

CLINICAL RATIONALE FOR THE REQUESTED PRODUCT

Velunexa is requested as the next line of therapy. Interleukin-23p19 inhibition is appropriate for a patient with widespread plaque disease refractory to conventional systemic therapy and phototherapy, and the every-twelve-week maintenance interval suits a patient whose adherence to weekly dosing has been a barrier. Tuberculosis and hepatitis screening are complete and negative, and there is no history of malignancy, active infection, or inflammatory bowel disease.

PRESCRIBER ATTESTATION

I certify that the information above is accurate, that I have personally examined this patient, and that Velunexa is medically necessary for the treatment of this condition.

PRESCRIBER SIGNATURE Dana Whitfield	TITLE MD	DATE 2026-08-24
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